

PFTs in Allergy & Asthma — Will You Get Paid?

Coverage · Frequency · Payment quick reference. CPT 94010 · 94060 · 94726 · 94729. 2025 Medicare national averages.

Will insurers pay for it?

Yes. Spirometry (94010), pre/post bronchodilator reversibility (94060), lung volumes (94726), and DLCO (94729) are routine, long established diagnostic codes, all performable in office with a portable PFT system. Medicare pays whenever medical necessity is documented in the chart, and the major commercial payers (UnitedHealthcare, Aetna, Cigna, BCBS, Humana) follow the same Medicare framework.

Is there a cap?

No hard cap, but a default. CMS coverage policy expects most PFTs about once per year. But asthma is, by definition, a chronic and variable disease, so objective testing is supported at diagnosis, at each meaningful change in control or therapy, and to confirm response. Guidelines recommend spirometry at least every 1 to 2 years in stable asthma and more often when control changes. A bronchial challenge (94070 / 95070) is payable when spirometry is normal but asthma is still suspected. Commercial plans typically want prior authorization above roughly 2 to 3 per year.

The bottom line for the chart

Pair the right ICD-10 (asthma type and severity, or the presenting symptom) with the visit and write one sentence of clinical rationale. Append modifier 25 to a same day office visit. That single habit is what gets the claim paid and protects it from a later audit.

What each code pays

CPT	Test	2025 Medicare avg.
94010	Spirometry	—
94060	Reversibility, pre/post bronchodilator (includes spirometry)	~\$37.85
94726	Lung volumes, TLC and RV by plethysmography	~\$54.99
94729	DLCO, add on (gas transfer / phenotyping)	~\$54.34
Full panel	94060 + 94726 + 94729, in office	~\$147

1. Confirmed asthma and airway diagnoses (recurring revenue)

Bill 94060 when you give a bronchodilator (it already includes the spirometry); bill 94010 alone for follow up spirometry. Add 94726 (lung volumes) and 94729 (DLCO) for a full panel. Never bill 94010 and 94060 on the same date.

Qualifying diagnosis	ICD-10	Bill (CPT)	Frequency you can support
Allergic asthma, mild to moderate persistent	J45.30/.31, J45.40/.41	94060 / 94010	At dx, each step change, 1 to 2x/yr stable; full panel when a biologic or alt dx is considered
Severe persistent / difficult to control	J45.40/.41, J45.50/.51	94060 / 94726 / 94729	More frequent monitoring; full panel incl. lung volumes + DLCO
Exercise induced / cough variant	J45.990/.991, J45.52	94060 / 94726 / 94729	Add methacholine challenge (94070 / 95070) when baseline spirometry is normal
Asthma–COPD overlap	J44.9 / J45.909	94060 / 94726 / 94729	Full PFT with DLCO to separate reversible obstruction from emphysema; periodic
Uncontrolled / recent exacerbation	J45.901 / J45.902	94060 / 94010	At each significant change in control or after an exacerbation

2. Symptom driven (no confirmed diagnosis needed)

Any patient with new dyspnea, wheeze, chronic cough, chest tightness, or exercise intolerance qualifies for a diagnostic PFT before a diagnosis is confirmed. Code the symptom as primary and add the known allergic condition (rhinitis, atopy) as secondary.

Presenting symptom	ICD-10	Bill (CPT)
Shortness of breath / dyspnea	R06.00	94060 / 94726 / 94729
Wheezing	R06.2	94060 / 94010
Chronic / unspecified cough	R05.3 / R05.9	94060 / 95070

Billed once per episode. If asthma is confirmed, the patient moves to Section 1 and becomes recurring revenue.

3. Therapy monitoring (controllers and biologics)

- **Controller step up.** Before and after starting or escalating ICS / LABA, bill 94060; objective pre/post response documents that the step up was warranted and is working.
- **Biologic candidate or on biologic.** Omalizumab, mepolizumab, dupilumab, benralizumab, tezepelumab. Asthma Dx + Z79.51; baseline and periodic PFT documents response; add FeNO (95012) to support T2 / eosinophilic phenotyping.

- **Chronic systemic steroid use.** Frequent OCS bursts or maintenance prednisone. Asthma Dx + Z79.52 / Z79.899; 94060 / 94726 / 94729 is the strongest case for a full panel.

CPT key

94010 spirometry · **94060** spirometry pre/post bronchodilator (reversibility, includes 94010) · **94726** lung volumes by plethysmography (use 94727 for gas dilution) · **94729** DLCO add on, must accompany spirometry or lung volumes · **94070** methacholine challenge · **95070** inhalation bronchial challenge · **95012** FeNO.

This is a coding and clinical guidance reference, not legal or billing advice. Coverage is determined by each patient's payer, plan, and Medicare Administrative Contractor (MAC). Verify with your local LCD before billing. Reimbursement amounts are CMS national averages for non facility settings and update annually.